# FLAIR Task Schematic

## Sepsis Prediction with Antibiotic-Anchored Time-Zero

FLAIR Benchmark

May 28, 2026

**Abstract**

We define a new FLAIR benchmark task that predicts *imminent first qualifying intravenous antibiotic administration* as an operational proxy for clinician recognition of sepsis. The task fixes time-zero ($t_{\text{zero}}$) at the first qualifying antibiotic event, starts predictions at the first recorded vital sign, and stops predictions strictly before $t_{\text{zero}}$ to avoid the label-leakage failure mode documented by Kamran et al. (2024). This document surveys ten prior operationalizations of sepsis onset, motivates the FLAIR choice, and specifies the labeling, cohort, and censoring rules with TikZ schematics. Input feature design is out of scope.

## 1 Introduction

Sepsis prediction is the most-studied AI use case in inpatient EHR research, yet there is no consensus on *when* a model's prediction should be considered correct (Singer et al., 2016; Rhee et al., 2019). The choice of time-zero changes both the label and the cohort, and Kamran et al. (2024) recently showed that the Epic Sepsis Model's AUROC drops from 0.62 to 0.47 when predictions made *after* a clinician's first sepsis-treatment order are excluded. This is the *label-leakage* problem: when the gold label is itself an administrative event (the antibiotic order), any prediction made after that order trivially correlates with the outcome it is supposed to forecast.

FLAIR's existing tasks (intubation, AKI, mortality, readmission) use clinical events that are either clearly involuntary (deterioration, death) or that occur at a clean state transition (ICU discharge). Sepsis is fundamentally different: the label is a clinician action. The task therefore must (i) pick an unambiguous anchor and (ii) censor predictions made at or after that anchor.

**We adopt the antibiotic-anchored framework**: $t_{\text{zero}}$ is the first qualifying systemic IV antibiotic, and the prediction window terminates strictly before $t_{\text{zero}}$. This mirrors the evaluation protocol of Kamran et al. (2024) translated into a *training label* rather than a post-hoc evaluation gate.

## 2 Survey of prior time-zero definitions

Table 1 summarizes ten prior definitions and what each implies for prediction-window start, stop, and label leakage.

Table 1: Operational time-zero definitions across major sepsis-prediction studies.

| Paper / model | Time-zero rule | Prediction horizon | Stop rule |
|---|---|---|---|
| Seymour et al. (2016) Sepsis-3 (JAMA) | $t_{\text{susp}}$ = earlier of {IV abx, blood culture} provided abx→cult $\leq$ 24h or cult→abx $\leq$ 72h; $t_{\text{sepsis}} = t_{\text{susp}} + (\Delta\text{SOFA} \geq 2$ in $(-48, +24)$h) | — (definition paper) | — |
| Reyna et al. (2020) PhysioNet/CinC 2019 | Seymour rule + IV abx sustained $\geq$72h; $t_{\text{sepsis}}$ = earlier of $t_{\text{susp}}, t_{\text{SOFA}}$ if $t_{\text{SOFA}} \in (-24, +12)$h of $t_{\text{susp}}$ | 6h ahead, hourly | onset or discharge |
| Desautels et al. (2016) InSight | $t_{\text{susp}}$ = earliest culture or abx; SOFA window $(-48, +24)$h | hourly, multi-horizon | onset |
| Shashikumar et al. (2021a) DeepAISE | Seymour rule; exclude sepsis in first 4h of ICU; ICU LOS 8h–20d | $T \in \{4, 6, 8, 12\}$h ahead | onset / discharge |
| Shashikumar et al. (2021b) COMPOSER | blood culture + $\geq$ 4d non-prophylactic IV abx, Seymour pairing | continuous | onset |
| Epic ESM(Wong et al., 2021) | proprietary; evaluated against Sepsis-3 | every 20 min throughout stay | discharge |
| Adams et al. (2022) TREWS | Sepsis-3 with provider confirmation | hourly until first abx or provider override | provider confirms / abx |
| **Kamran et al. (2024) (user-cited)** | **Label** = composite of CDC ASE (Rhee et al., 2019) and CMS SEP-1 (Centers for Medicare and Medicaid Services, 2015). **Evaluation gate**: predictions are restricted to time-points strictly *before* the first "treatment indicator" timestamp $\in$ {antibiotics, blood culture, lactate, IV fluids}. *No fixed $\Delta$-gate* — the indicator timestamp itself is the cutoff. | ESM scores every 20 min throughout admission | min(first treatment indicator, sepsis criteria met, death, discharge) |
| Rhee et al. (2019) CDC ASE | $\geq$ 4d qualifying abx within $\pm$2d of blood culture + eSOFA organ dysfunction | calendar-day | — |
| Centers for Medicare and Medicaid Services (2015) CMS SEP-1 | 2 SIRS criteria + 1 organ dysfunction within 6h of one another | — | — |

**Implications for FLAIR.** Three patterns emerge from Table 1. First, the Seymour-paired definition dominates the AI-sepsis literature but is sensitive to culture order timing, which itself reflects clinician suspicion. Second, no two studies use the same cohort entry rule: ICU-only (Desautels et al., 2016; Shashikumar et al., 2021a) vs. hospital-wide (Kamran et al., 2024; Wong et al., 2021) vs. ED-onset (Henry et al., 2022) produce materially different prevalence estimates. Third, Kamran et al. (2024) is the first study to *quantify* the cost of label leakage by re-evaluating an existing deployed model under a stricter "before clinician action" rule. We adopt their pre-treatment-indicator gate as a structural property of the training label itself.

# 3 FLAIR operational definition

## 3.1 Anchors and windows

Let $H_i$ denote a hospitalization with admission time $t_i^{\text{adm}}$ and discharge or death time $t_i^{\text{end}}$.

**Time-zero ($t_{\text{zero}}$).**
$t^i_{\text{zero}} = \min\{\, t \mid \text{med_admin}(i,t) \in \mathcal{A}_{\text{IV-systemic}} \,\}$ where $\mathcal{A}_{\text{IV-systemic}}$ is the curated systemic-IV-antibiotic drug list (excludes topicals, prophylaxis single-doses, oral abx).

**Prediction-window start (pred_start).**
$\text{pred_start}_i = \min\{\, t \mid \text{vitals}(i,t) \neq \emptyset \,\}$ — the first recorded vital sign per hospitalization. Floor at $t^{\text{adm}}_i + 1\text{h}$ to guarantee a non-degenerate window when vitals are taken immediately at admission.

**Prediction-window stop (pred_stop).**
$\text{pred_stop}_i = \min(t^i_{\text{zero}},\ t^{\text{end}}_i)$. The interval $[\text{pred_start}_i, \text{pred_stop}_i)$ is closed-open; $t^i_{\text{zero}}$ itself is *not* a decision point.

**Decision-point grid.**
Hourly grid: $D_i = \{\text{pred_start}_i + k\cdot 1\text{h} \ :\ k \in \mathbb{Z}_{\geq 0},\ \text{pred_start}_i + k\cdot 1\text{h} < \text{pred_stop}_i\}$.

**Label (`label_sepsis_abx_6h`).**
For $d \in D_i$:

$$y_i(d) = \mathbb{1}\Big[t^i_{\text{zero}} \in (d,\ d + 6\text{h}]\Big].$$

Patients with no qualifying antibiotic ever ($t^i_{\text{zero}} = +\infty$) contribute all-negative rows over the entire $[\text{pred_start}_i, t^{\text{end}}_i)$.

### 3.2 Cohort

Hospital-wide, mirroring Kamran et al. (2024):

- Age at admission $\geq 18$ with valid `admission_dttm` and `discharge_dttm`.
- Exclude hospitalizations on psychiatric or rehabilitation units (Kamran 2024, Methods).
- Exclude $t^i_{\text{zero}} \leq \text{pred_start}_i$ (admitted-with-treatment; Kamran 2024: "patients ... who received treatment before the first measurement of vital signs").
- Exclude hospitalizations with no vital-sign measurements (cannot anchor pred_start).

### 3.3 Sensitivity arms (documentation only, not primary output)

- **ICU-only arm.** Restrict to hospitalizations with $\geq 1$ ICU ADT record; $\text{pred_start}_i = \max(\text{first_icu_start}_i + 1\text{h}, \text{first vital})$.
- **Seymour-paired arm.** Require blood culture in $(t_{\text{zero}} - 72\text{h},\ t_{\text{zero}} + 24\text{h})$ for the antibiotic to qualify.
- **Sepsis-3 label arm.** Replace abx-as-label with $\Delta\text{SOFA} \geq 2$ within $\pm 48\text{h}$ of $t_{\text{susp}}$ (PhysioNet 2019 rule) for direct cross-comparison.

# 4 Algorithmic flow

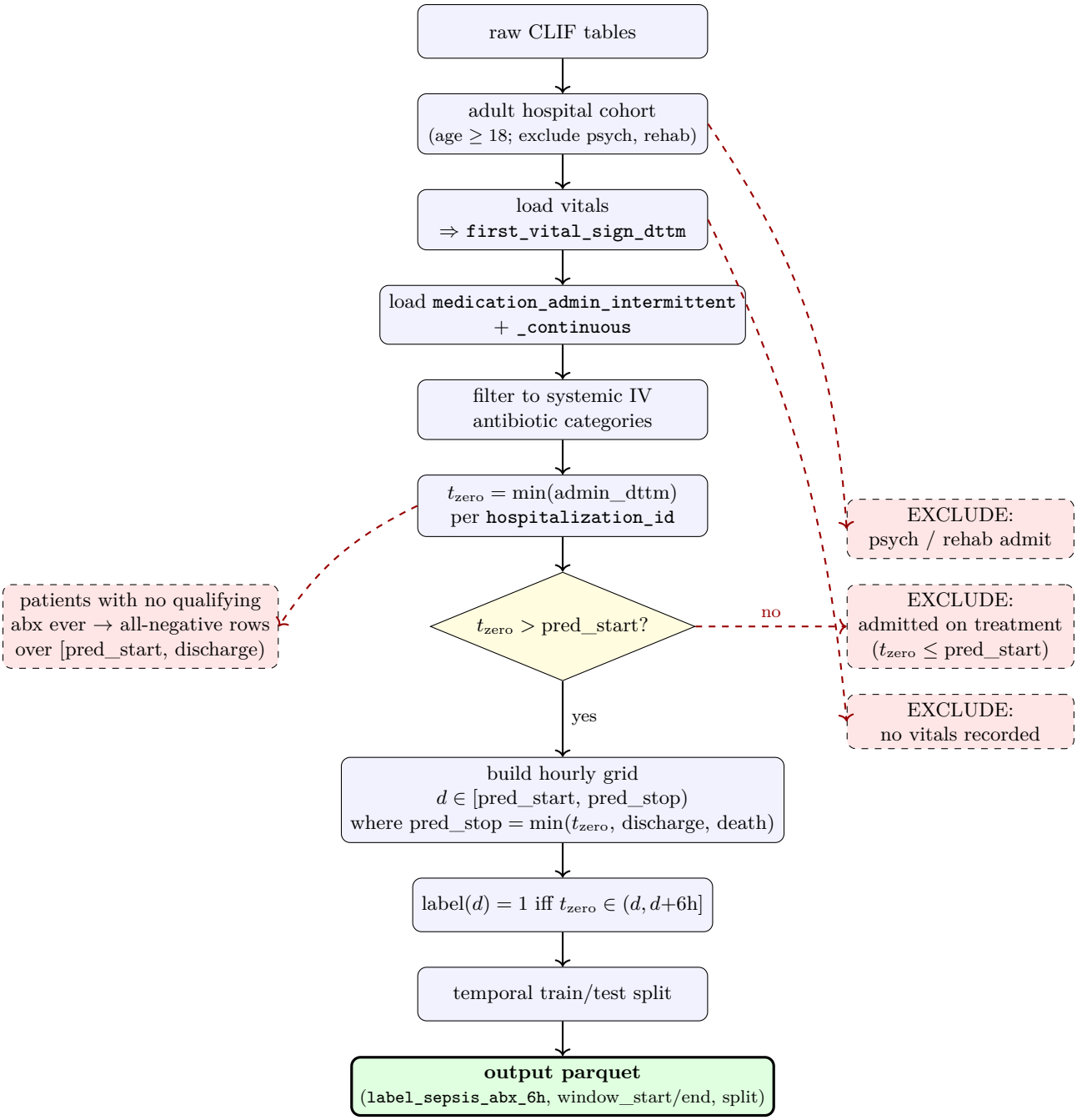



Figure 1: Build pipeline. Solid arrows: main flow. Dashed red arrows: exclusions. The diamond enforces the label-leakage gate: any hospitalization whose qualifying antibiotic precedes its first vital sign is dropped, since by Kamran et al.'s argument no decision point in that stay can produce a recognition-free prediction.

Step-by-step prose:

1. Load CLIF tables: `hospitalization`, `patient`, `adt`, `vitals`, and both `medication_admin_intermittent` / `medication_admin_continuous`.
2. Apply hospital-wide cohort filters: age, psych/rehab exclusion.
3. Compute pred_start$_i$ from `vitals.recorded_dttm`.
4. Filter `medication_admin_*` to $\mathcal{A}_{\text{IV-systemic}}$; compute $t^i_{\text{zero}} = \min(\text{admin_dttm})$ per `hospitalization_id`.
5. Apply the leakage gate: drop $t^i_{\text{zero}} \leq \text{pred_start}_i$.
6. Explode the hourly decision-point grid [pred_start$_i$, pred_stop$_i$) per hospitalization (`pl.datetime_ranges`, closed-open).
7. Assign $y_i(d)$ per the label rule.
8. Assign `window_start` $= d - 24$h, `window_end` $= d$ (lookback window, fixed 24h to match the FLAIR convention).
9. Apply temporal train/test split per `flair_benchmark._split.assign_split`.
10. Select `OUTPUT_COLS` and persist as parquet.

## 5 Timeline diagram

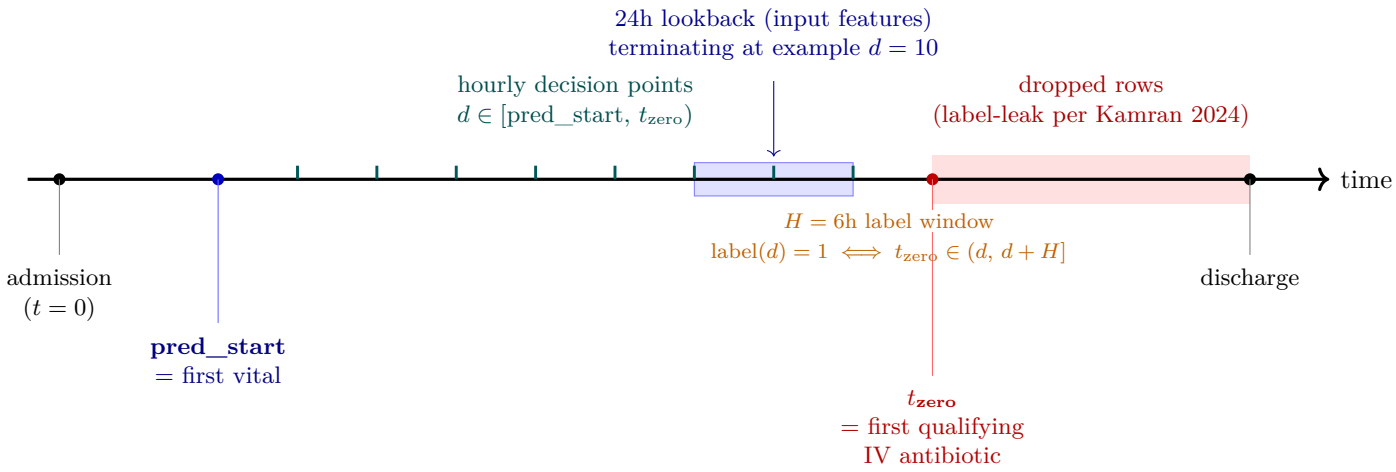



Figure 2: Hospitalization timeline. The blue "lookback" bar shows the 24h rolling feature window terminating at one example decision point. The red "dropped rows" zone covers $[t_{\text{zero}}, \text{discharge})$ — any prediction made at or after the antibiotic order is removed from training and evaluation, eliminating the label-leakage channel identified by Kamran et al. (2024).

## 6 Design choices and rationale

**Why antibiotic-as-anchor (not SOFA $\Delta \geq 2$).** The clinical workflow this task targets is the moment a clinician decides to treat for suspected infection. SOFA-anchored labels (Singer et al.,

2016; Reyna et al., 2020) describe physiology, not action: a model that predicts SOFA deterioration may be operationally useless if the deterioration is recognized faster than the model's lead time. Antibiotic order is the action we want to bring forward.

**Why strictly pred_stop = $t_{\text{zero}}$ with no Δ-gate.** Kamran et al. (2024) do not introduce a fixed buffer; they restrict analysis to "predictions occurring before sepsis criteria were met and before the first indicator of treatment." Adding a buffer is an additional design choice and is documented in Table 1 for methods that adopt one. FLAIR follows the parsimonious Kamran rule: the order timestamp itself is the cutoff.

**Why hospital-wide (not ICU-only).** Hospital-wide matches Kamran et al. (2024)'s cohort and captures the operationally important ward-onset sepsis cases, where prediction lead time has the most clinical value. The ICU-only variant is preserved as a sensitivity arm for comparability with Shashikumar et al. (2021a) and Desautels et al. (2016).

**Why hourly grid.** Matches the FLAIR convention (task5 AKI-24h uses the same hourly grid) and the PhysioNet/CinC 2019 standard. ESM's native 20-minute cadence (Wong et al., 2021; Kamran et al., 2024) is finer than necessary for benchmarking and would inflate row counts ∼3× without adding label resolution.

**Prevalence implications.** Hospital-wide hourly grids produce highly imbalanced labels: a 7-day stay with one antibiotic event produces ≤ 6 positive rows out of ∼160. Reporting AUPRC alongside AUROC is mandatory, per the `META.evaluation_metrics` convention.

**Drug list.** The systemic-IV-antibiotic curation is the largest open dependency and is deferred to a follow-up round. CLIF's `med_category` mapping must be extended with an `antibiotic_systemic_iv` class. The list should follow Rhee et al. (2019)'s "qualifying antimicrobial therapy" definition (CDC ASE Appendix).

## 7 Out of scope

- Implementation of `flair_benchmark/tasks/task4_sepsis_abx_6h.py`.
- Curated antibiotic drug list and `med_category` mapping.
- Choice of input features and feature engineering.
- Sensitivity-arm output schemas.

## References

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